Executive Summary

The Phoenix Indian Medical Center (PIMC) is committed to providing high-quality, culturally competent healthcare services to American Indian and Alaska Native (AI/AN) patients. In order to ensure that the PIMC's policies and procedures are consistent with its mission, values, and strategic plan, the PIMC has established a Policy Committee.

The Policy Committee is an advisory body to the PIMC Chief Quality Officer (CQO) and is composed of representatives from various departments, as well as AI/AN community representatives. The committee is responsible for reviewing all proposed policies and procedures, identifying and recommending revisions to existing policies and procedures, and providing guidance to the PIMC leadership on policy matters.

A Policy Committee is essential for the PIMC for the following reasons:

- **Ensures that policies and procedures are consistent with the PIMC's mission, values, and strategic plan.** The Policy Committee reviews all proposed policies and procedures to ensure that they are aligned with the PIMC's overall goals and objectives.
- **Ensures that policies and procedures are culturally competent.** The Policy Committee includes AI/AN community representatives who can provide input on the cultural appropriateness of policies and procedures.
- **Identifies and recommends revisions to existing policies and procedures.** The Policy Committee reviews existing policies and procedures on a regular basis to identify any areas that need to be updated or revised.
- **Provides guidance to the PIMC leadership on policy matters.** The Policy Committee can provide valuable feedback to the PIMC leadership on policy issues.
- **Ensures that policies and procedures are compliant with regulatory agencies.** The Policy Committee reviews all proposed policies and procedures to ensure that they are aligned with relevant laws, regulations, and standards.

In conclusion, a Policy Committee is essential for the PIMC to ensure that its policies and procedures are of the highest quality. The Policy Committee helps to ensure that the PIMC is providing high-quality, culturally competent healthcare services to AI/AN patients.

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Purpose

The purpose of the Policy Committee is to develop, review, and recommend policies for the hospital. The Phoenix Indian Medical Center Policy Committee will ensure that all policies are consistent with the hospital's mission, vision, and values and comply with all applicable laws and regulations. The committee will review all policies and procedures related to clinical care and operations at the Phoenix Indian Medical Center including the hospital, and the outpatient/ambulatory clinics.

Definitions:

Electronic repository: refers to the system or database where the policy documents are stored. This could be a file share, a document management system, or another type of electronic storage system

Policy: A statement of principles and values that directs decision-making and/or helps the organization achieve its objectives and maintain compliance. Aligns and communicates behavioral expectations for the workforce (clinical or administrative). Is concise, broad in scope, high level, and is required to be followed.

Procedure/Standard Operating Procedure (SOP): A detailed, step-by-step description of how to practice compliantly according to policy or to achieve a specific outcome. Aligns and communicates behavioral expectations for the workforce (clinical or administrative). Provides details on how to practice compliantly according to policy or to achieve a specific outcome. May be clinical or administrative. Explains the responsibilities and practical steps involved for the workforce. Is detailed, may be of broad application, or may be localized to reflect practices of a care population, specific clinical condition, or environment, and must be followed.

Plan: A framework for change to accomplish objectives, addressing the scope, resources, and schedule to meet those objectives. A plan includes risk assessment and mitigation activities, describes the responsibilities of individuals and groups, and provides measures of success. Not maintained in the electronic repository.

Operational Standard: Describes the level of quality or attainment to meet a policy, guideline, or operational need. Must be followed.

Clinical Protocol: Describes a system of actions that directs a clinical course in specifically defined situations or treatments. Localizes the expectations of a clinical guideline to our clinical care environment. An order must be placed to initiate the protocol for non-privileged providers unless there is an approved standing order. Deviation may be allowed within the scope of practice, but only under the specific circumstances/situations defined in the protocol.

Standing Orders: A predefined set of approved orders that may be initiated by a non-privileged provider if the patient meets certain clinically defined criteria. Must be reviewed annually.

Order Sets: A list of individually selectable approved interventions or orders that the clinical staff is authorized to use. Not maintained in the electronic repository.

Guidance Documents: Documents that provide guidance and require clinical/professional judgment in their application. Not maintained in the electronic repository.

Guideline: A set of rules or behaviors that set standards and/or determine a course of action. Not maintained in the electronic repository.

Clinical Guidelines (or CPGs): Clinical Practice Guidelines refer to tools that describe the expected best practice to reach a particular clinical goal or manage a particular clinical condition. Defines what care is appropriate and why. Often represents evidence-based best practice or the cumulative wisdom of the

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clinical field, and may be broad guidance across organizations. Judgment is required to determine whether a clinical guideline applies to a particular patient or situation; but if it does, it is expected the guideline would be followed. Not maintained in the electronic repository.

Operational Guidelines: Provides guidance and expected best practice relative to a particular topic or area of operations. Often represents the cumulative wisdom of the organization. Operational guidelines are the administrative equivalent to clinical guidelines. Not maintained in the electronic repository.

Duplication of policies: refers to the situation where two or more policy documents contain the same or overlapping content. This can lead to confusion and inconsistency, and it can also make it difficult to find the most up-to-date information.

Related entity/facility/system documents: refers to other policy documents that are related to the document being reviewed. This could include documents that are specific to the same entity, facility, or system, or documents that address similar topics.

Policy/Purpose Statement: A summary statement of the overarching principles that guide a specific policy, procedure, plan or guideline. A policy or purpose statement is recorded on each Document under the "Policy or Purpose Statement" header. It is a statement of the Document's intent.

Guest/End-User: Any employee or affiliate who uses the electronic policy repository to search and view active Documents.

Owner or designee refers to the person who is responsible for the policy document. This could be the author of the document, the manager of the department or team that created the document, or another designated individual. An employee or affiliate assigned to shepherd a Document through its creation, review, and/or revision process. The Owner's name appears in the top right header of each Document.

Membership

The Policy Committee will be composed of the following members:

- Five PIMC employees, including one representative from each of the following departments: clinical services, administrative services, support services, and community relations.
- Two AI/AN community representatives.
- One representative from the PIMC Tribal Advisory Council.
- One representative from the PIMC Employee Council.
- One Policy and Procedure consultant

Roles and Responsibilities

The Policy Committee will have the following roles and responsibilities:

- Review updated existing policies and procedures on a regular basis.
- Ensure that all policies and procedures are consistent with the hospital's mission, vision, and values and comply with all applicable laws and regulations.
- Ensure training and education plans are in place to train and educate staff members on new and revised policies and procedures.
- Monitor the implementation and effectiveness of all policies and procedures.
- Make recommendations to the hospital's executive leadership team on any changes to policies or procedures.

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Meeting Schedule

The Policy Committee will meet on a quarterly basis, or more frequently as needed.

Decision-Making Process

The Policy Committee will make decisions by consensus. If consensus cannot be reached, the majority vote of the Committee members will prevail.

Approval Process

All new and revised policies and procedures must be approved by the Policy Committee

Roles and Responsibilities for Policy Committee Members

Chair

- Preside over Policy Committee meetings.
- Establish and maintain the agenda for Policy Committee meetings.
- Facilitate discussion and decision-making among Policy Committee members.
- Ensure that all Policy Committee decisions are documented and communicated to affected staff members.

Members

- Participate actively in Policy Committee meetings and discussions.
- Provide input and feedback on new and revised policies and procedures.
- Review and approve all new and revised policies and procedures.
- Communicate new and revised policies and procedures to affected staff members.
- Provide training and education to staff members on new and revised policies and procedures.
- Monitor the implementation and effectiveness of all policies and procedures.
- Make recommendations to the Policy Committee on any changes to policies or procedures that are needed.

The Policy Committee is an important part of the hospital's governance structure. By developing, reviewing, and recommending policies, the Committee helps ensure that the hospital operates safely, efficiently, and competently.

Effective Date

This policy is effective immediately.

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Bylaws of the Phoenix Indian Medical Center Policy Committee

Article I - Name

The name of the committee shall be the Phoenix Indian Medical Center Policy Committee (PIMC Policy Committee).

Article II - Purpose

The purpose of the PIMC Policy Committee is to:

- Develop, Review and recommend policies and procedures that are consistent with the PIMC's mission, values, and strategic plan.
- Ensure that the PIMC's policies and procedures are culturally competent and responsive to the needs of AI/AN patients.
- Review and recommend revisions to existing PIMC policies and procedures.
- Provide guidance to the PIMC leadership on policy matters.

Article III - Membership

Section 1. Composition.

The PIMC Policy Committee will be composed of the following members:

- Five PIMC employees, including one representative from each of the following departments: clinical services, administrative services, support services, and community relations.
- Two AI/AN community representatives.
- One representative from the PIMC Tribal Advisory Council.
- One representative from the PIMC Employee Council.
- One Policy and Procedure consultant

The Policy and Procedure consultant will provide expert advice and guidance to the committee on the development, implementation, and evaluation of PIMC policies and procedures.

The Policy and Procedure consultant will be responsible for the following:

- Conducting research and analysis on policy issues.
- Assisting in the drafting and revising policies and procedures.
- Ensuring that training and education on PIMC policies and procedures is coordinated through Staff Development.
- Monitoring the implementation of PIMC policies and procedures.

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- Evaluating the effectiveness of PIMC policies and procedures.
- Evaluating policies for clarity and congruency.

Section 2. Terms of Office.

Members of the PIMC Policy Committee shall serve three-year terms. Terms shall be staggered so that one-third of the members are elected each year.

Section 3. Vacancies.

Vacancies on the PIMC Policy Committee shall be filled for the unexpired term by the PIMC CQO, in consultation with the appropriate appointing authority.

Section 4. Removal from Office.

A member of the PIMC Policy Committee may be removed from office for cause by the PIMC CEO, in consultation with the appropriate appointing authority.

Article IV - Meetings

Section 1. Meetings.

The PIMC Policy Committee shall meet quarterly, or more often as needed.

Section 2. Notice of Meetings.

Notice of all meetings of the PIMC Policy Committee shall be given at least ten days in advance of the meeting.

Section 3. Quorum.

A majority of the members of the PIMC Policy Committee (6 of 10) shall constitute a quorum for the transaction of business.

Section 4. Voting.

Each member of the PIMC Policy Committee shall have one vote.

Section 5. Parliamentary Procedure.

Robert's Rules of Order shall govern the proceedings of the PIMC Policy Committee.

Article V - Officers

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Section 1. Officers.

The PIMC Policy Committee shall elect a chair and vice chair from among its members at its first meeting each year.

Section 2. Duties of the Chair.

The chair shall preside at all meetings of the PIMC Policy Committee and shall perform such other duties as may be assigned by the committee.

Section 3. Duties of the Vice Chair.

The vice chair shall perform the duties of the chair in the chair's absence or disability.

Article VI - Committees

The PIMC Policy Committee may establish such subcommittees as it deems necessary to carry out its work.

Article VII - Records

The PIMC Policy Committee shall maintain minutes of all its meetings.

Article VIII - Amendments

These bylaws may be amended by a two-thirds vote of the PIMC Policy Committee. Amendments must be approved by the PIMC CQO.

Article IX - Effective Date

These bylaws are effective immediately upon approval by the PIMC CQO.

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Charter for the Phoenix Indian Medical Center Policy Committee

Preamble

The Phoenix Indian Medical Center (PIMC) is committed to providing high-quality, culturally competent healthcare services to American Indian and Alaska Native (AI/AN) patients. The PIMC Policy Committee is established to ensure that the PIMC's policies and procedures are consistent with the PIMC's mission, values, and strategic plan.

Mission

The mission of the PIMC Policy Committee is to:

- Assist in the development, review and recommendation policies and procedures that are consistent with the PIMC's mission, values, and strategic plan.
- Ensure that the PIMC's policies and procedures are culturally competent and responsive to the needs of AI/AN patients.
- Review and recommend revisions to existing PIMC policies and procedures.
- Provide guidance to the PIMC leadership on policy matters.

Values

The PIMC Policy Committee will adhere to the following values:

- Respect for AI/AN culture and traditions.
- Patient-centered care.
- Quality and safety.
- Collaboration and teamwork.
- Transparency and accountability.

Authority

The PIMC Policy Committee is an advisory body to the PIMC Chief Quality Officer (CQO). The PIMC CQO has the final authority to approve or disapprove all policies and procedures recommended by the committee.

Composition

The PIMC Policy Committee will be composed of the following members:

- Five PIMC employees, including one representative from each of the following departments: clinical services, administrative services, support services, and community relations.
- Two AI/AN community representatives.
- One representative from the PIMC Tribal Advisory Council.

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- One representative from the PIMC Employee Council.
- One Policy and Procedure consultant

The Policy and Procedure consultant will provide expert advice and guidance to the committee on the development, implementation, and evaluation of PIMC policies and procedures.

The Policy and Procedure consultant will be responsible for the following:

- Conducting research and analysis on policy issues.
- Assisting in the drafting and revising policies and procedures.
- Ensuring that training and education on PIMC policies and procedures is coordinated through Staff Development..
- Monitoring the implementation of PIMC policies and procedures.
- Evaluating the effectiveness of PIMC policies and procedures.
- Evaluating policies for clarity and congruency.

The Policy and Procedure consultant will be a valuable asset to the PIMC Policy Committee and will help to ensure that the PIMC's policies and procedures are of the highest quality.

The PIMC CQO will serve as an ex officio member of the committee.

Meetings

The PIMC Policy Committee will meet quarterly, or more often as needed. The committee will elect a chair and vice chair at its first meeting.

Responsibilities

The PIMC Policy Committee will be responsible for the following:

- Reviewing proposed policies and procedures for consistency with the PIMC's mission, values, and strategic plan.
- Identifying and recommending revisions to existing PIMC policies and procedures.
- Providing feedback on proposed policies and procedures to the PIMC leadership.
- Developing and recommending policies and procedures that are culturally competent and responsive to the needs of AI/AN patients.
- Providing guidance to the PIMC leadership on policy matters.

Reporting

The PIMC Policy Committee will report to the PIMC CQO on a quarterly basis.

Amendments

This charter may be amended by a two-thirds vote of the PIMC Policy Committee.

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Amendments must be approved by the PIMC CQO.

Effective Date

This charter is effective immediately upon approval by the PIMC CQO.

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Purpose:

The Phoenix Indian Medical Center (PIMC) maintains an electronic repository for the management of policies, procedures, plans and standards. This policy serves to outline the process for document review, management, and updates. The policy clearly defines the responsibilities of various roles, such as Document Editors, Owners, Approvers, Policy Area Owners, and Site Administrators. Permissions are assigned to different roles based on their organizational responsibilities, ensuring that the appropriate individuals have access to the relevant documents. The definitions below were developed to inform clinical policy governance work.

Definitions:

Electronic repository: refers to the system or database where the policy documents are stored. This could be a file share, a document management system, or another type of electronic storage system

Policy: A statement of principles and values that directs decision-making and/or helps the organization achieve its objectives and maintain compliance. Aligns and communicates behavioral expectations for the workforce (clinical or administrative). Is concise, broad in scope, high level, and is required to be followed.

Procedure/Standard Operating Procedure (SOP): A detailed, step-by-step description of how to practice compliantly according to policy or to achieve a specific outcome. Aligns and communicates behavioral expectations for the workforce (clinical or administrative). Provides details on how to practice compliantly according to policy or to achieve a specific outcome. May be clinical or administrative. Explains the responsibilities and practical steps involved for the workforce. Is detailed, may be of broad application, or may be localized to reflect practices of a care population, specific clinical condition, or environment, and must be followed.

Plan: A framework for change to accomplish objectives, addressing the scope, resources, and schedule to meet those objectives. A plan includes risk assessment and mitigation activities, describes the responsibilities of individuals and groups, and provides measures of success. Not maintained in the electronic repository.

Operational Standard: Describes the level of quality or attainment to meet a policy, guideline, or operational need. Must be followed.

Clinical Protocol: Describes a system of actions that directs a clinical course in specifically defined situations or treatments. Localizes the expectations of a clinical guideline to our clinical care environment. An order must be placed to initiate the protocol for non-privileged providers unless there is an approved standing order. Deviation may be allowed within the scope of practice, but only under the specific circumstances/situations defined in the protocol.

Standing Orders: A predefined set of approved orders that may be initiated by a non-privileged provider if the patient meets certain clinically defined criteria. Must be reviewed annually.

Order Sets: A list of individually selectable approved interventions or orders that the clinical staff is authorized to use. Not maintained in the electronic repository.

Guidance Documents: Documents that provide guidance and require clinical/professional judgment in their application. Not maintained in the electronic repository.

Guideline: A set of rules or behaviors that set standards and/or determine a course of action. Not

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maintained in the electronic repository.

Clinical Guidelines (or CPGs): Clinical Practice Guidelines refer to tools that describe the expected best practice to reach a particular clinical goal or manage a particular clinical condition. Defines what care is appropriate and why. Often represents evidence-based best practice or the cumulative wisdom of the clinical field, and may be broad guidance across organizations. Judgment is required to determine whether a clinical guideline applies to a particular patient or situation; but if it does, it is expected the guideline would be followed. Not maintained in the electronic repository.

Operational Guidelines: Provides guidance and expected best practice relative to a particular topic or area of operations. Often represents the cumulative wisdom of the organization. Operational guidelines are the administrative equivalent to clinical guidelines. Not maintained in the electronic repository.

Duplication of policies: refers to the situation where two or more policy documents contain the same or overlapping content. This can lead to confusion and inconsistency, and it can also make it difficult to find the most up-to-date information.

Related entity/facility/system documents: refers to other policy documents that are related to the document being reviewed. This could include documents that are specific to the same entity, facility, or system, or documents that address similar topics.

Policy/Purpose Statement: A summary statement of the overarching principles that guide a specific policy, procedure, plan or guideline. A policy or purpose statement is recorded on each Document under the "Policy or Purpose Statement" header. It is a statement of the Document's intent.

Guest/End-User: Any employee or affiliate who uses the electronic policy repository to search and view active Documents.

Owner or designee refers to the person who is responsible for the policy document. This could be the author of the document, the manager of the department or team that created the document, or another designated individual. An employee or affiliate assigned to shepherd a Document through its creation, review, and/or revision process. The Owner's name appears in the top right header of each Document.

Responsibilities:

The Owner is responsible for ensuring:

A. The document content is accurate, reflects best practice standards, and complies with current regulations.
B. The document undergoes review according to its assigned review cycle.
C. All appropriate stakeholders have been identified and allowed to provide input into the document's creation/revision.
D. All stakeholder input, requests for revision(s), and approval are recorded and retained in the document history.
E. Approved headers and formats available within the system are used consistently throughout the document.
F. New/revised forms associated with the document that are considered part of the electronic medical record (EMR) and include a provider, patient, and/or caregiver signature line have been reviewed/approved by the Forms Committee of the Health Information Management Department.
G. The document impact assessment is completed/validated to establish the minimum education for the target audience.

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H. The system document education plan form is completed to reflect both minimum and additional education to be provided.
I. The completed system document education plan form is submitted to the Educational Functional Group for review/approval.
J. Any education plan requiring a system-wide CBL must be presented to and approved by the Executive Committee.
K. The approved document education plan is copied into or electronically retained in the policy history.
L. All education is implemented before the document is posted.
M. The policy site administrator is notified to post the document upon completing the approved education plan.
N. The policy site administrator is notified of any documents that need to be retired upon posting the new/revised document.
O. Any superseded documents are maintained on file per the retention of records policy.

Editor: An employee or affiliate responsible for creating and revising document content with the Owner's guidance.

Stakeholder: Any process owner with direct interest, involvement, or accountability to the subject covered in the Document that provides input and/or content approval. Stakeholders may include hospital committees, service lines, and/or physician committee chairpersons, hospital leaders, and/or one or more departments or their respective representatives. Stakeholders may also be individuals/groups in the Electronic Signature Approval Workflow. **The input/approval of ALL stakeholders is recorded and retained in the Document history.**

Approver: An employee/committee determined to be responsible for providing an Electronic Signature on a Document. These individuals/committees are listed in the Document's Electronic Approval Workflow. The number of individuals/committees in an Electronic Approval Workflow should be limited to two or three to facilitate the Document's movement through the electronic signature approval process. Any other individuals/groups considered to have a role in the Document's development/approval are considered Stakeholders, and their input/approval of the Document is recorded in the Document history. The final approver may not be the policy owner.

Policy Area Owner: An employee or affiliate who manages a Policy Area or Department. Policy Area Owners have author/owner permissions for one or more Policy Areas/Departments as appropriate to their organizational responsibility. In addition to having permission to edit/create Documents in specific Policy Areas, Policy Area Owners also receive reports of Documents approaching the end of their review life cycle. They can view pending Documents to facilitate the management of Documents within their assigned Policy Area(s).

Policy Site Administrator: An employee or affiliate assigned the highest level of access within the electronic repository responsible for setting user permissions within the program. The facility/Entity Site administrator works with leadership to maintain accurate user lists and permission levels and to monitor the facility/entity's adherence to Document review cycles. Policy Site Administrators develop/maintain competency in using the electronic repository program and serve as Super Users to the organization. Policy Site Administrators assist with technical issues according to ability and refer all the other problems to the vendor support desk.

NOTE: **The PIMC IT Department does NOT provide support for the electronic repository.**

Policy

All PIMC policies will be maintained in an electronic repository. Owners, stakeholders, approvers, editors, and site administrators are responsible for adhering to the procedures outlined in this policy.

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Procedure:

The following procedural steps identify the elements captured during the creation/review of a policy, procedure, plan, or non-clinical practice guideline.

Document Necessity (only applies to current Documents under review)

A. When a current Document is under review, the assigned Owner collaborates with the Stakeholders to determine if the Document is still necessary. If deemed necessary, the Owner continues the review/revision process.
B. If the Document is no longer necessary,
C. The Owner completes policy retirement form and contacts the appropriate Policy Site Administrator to retire it.

Identification/Assignment of a Document Owner

A. As a Document approaches the end of its review life cycle, the currently assigned Owner receives electronic notification of the approaching review date at 90, 60, and 30 days before the end. The Owner works with Stakeholders and appropriate leadership to determine if they are still the most logical Document Owner.
B. When the need for a new Document arises, appropriate Leadership/Stakeholders identify and assign the most logical and appropriate Owner based on the availability of content experts and/or other facility work teams.
C. The Owner completes the Document review/revision process according to its review life cycle.
D. Requests to re-assign an Owner must be approved by the Area owner.

Review of Existing Related Documents and Education Resources

The owner or designee of both new and current documents is responsible for:

A. Reviewing the electronic repository for related entity/facility/system documents.
B. Notifying the appropriate owners of any anticipated impact.
C. Preventing the duplication of policies.

In addition, the owner is responsible for:

A. Contact the Educational Functional Group to determine what Computer Based Learning (CBLs) and other education resources may be impacted by the development/revision of the document.
B. Notifying the appropriate owner(s) of identified educational resource(s) of any anticipated impact.

Drafts

A. All drafts older than 18 months will be deleted unless shown as being modified in the previous 12 months.

B. The draft owner will be contacted before the draft is deleted.

Title and Content

A. Titles and content are determined by consensus of the Owner and Stakeholders.
B. The electronic repository automatically assigns a unique identification number to each Document; therefore, no additional numbers are given.
C. Avoid the use of abbreviations in Document titles.

Policy Area

A. Each Document is assigned a Policy Area which describes where the document is applicable.
B. The Owner reviews the Policy Area assignment for appropriateness.
    1. If the Policy Area assignment is incorrect, the Owner contacts the appropriate

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Stakeholder(s), seeks input, and reaches consensus for a new Policy Area assignment.
2. When creating a new document, the Owner designates the Policy Area that most accurately reflects the department or service line to which the content pertains.
3. The Owner must contact the Policy Site Administrator for assistance if the correct Policy Area needs to be found in the approved repository menu.

Approval Workflow

A. An Approval Workflow identifies the employee(s) or committee(s) responsible for providing an Electronic Signature on the Document. These individuals or committees are listed in the Document's Electronic Approval Workflow and displayed at the bottom.
B. **Each Approval Workflow is given a unique title, and the first step (Approval Step) is assigned to the Document Owner.**
C. **The individuals named in an Approval Workflow must have an email account and actively utilize this email account.**
D. An employee or affiliate is typically assigned as a designee for Physician Approvers.
E. When a group/committee is named in an Approval Workflow, one member is assigned to perform the Electronic Signature to represent the group/committee.
F. The number of individuals/groups in an Electronic Approval workflow should be limited to two or three to facilitate the Document's movement through the approval process.
G. All other individuals or groups considered to have a role in the policy's development and approval are considered Stakeholders, and their input and support of the Document is retained in the policy history.
H. **The employee(s)/committee(s) in the Electronic Signature Approval Workflow are often also Stakeholders in the policy. Therefore, the Owner ensures their input and content approval is collected before initiating the Electronic Signature Approval Workflow.**
    1. When reviewing a current Document, the Owner collaborates with Stakeholders to determine if the assigned Approval Workflow is correct.
        a. If correct, the Owner continues the review/revision process
        b. If incorrect, insert the correct Approval Workflow
I. When creating a new Document, the Owner assigns the correct Approval Workflow in collaboration with the Policy Area Owner
J. All workflows will have a policy committee representative in the approval workflow process.
K. If the Approval Workflow for a new or current Document is not found in the repository menu, please contact the Policy Site Administrator to create a new workflow.

Review Date

A. The standard review cycle for policies, procedures, plans, and non-clinical practice guidelines is three years or 1095 days unless otherwise specified by a regulatory body or leadership.
B. The standard review for Standing Orders is 365 days.
C. For current Documents under review, the Owner determines if the existing review life cycle length is appropriate and adjusts accordingly.
D. For new Documents, the Owner works with appropriate Stakeholders/Leaders to determine the proper review life cycle length with consideration of regulatory standard(s) requirement(s).
E. **All Documents in the live repository are active until superseded by another Document or retired.**

Policy Committee

The Phoenix Indian Medical Center (PIMC) is committed to providing high-quality, culturally competent healthcare services to American Indian and Alaska Native (AI/AN) patients. To ensure that the PIMC's policies and procedures are consistent with the PIMC's mission, values, and strategic plan, the PIMC has established a Policy Committee.

The Policy Committee is an advisory body to the PIMC Chief Quality Officer (CQO) and is composed of representatives from various departments, as well as AI/AN community representatives. The committee is responsible for reviewing all proposed policies and procedures, identifying and recommending revisions

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to existing policies and procedures, and providing guidance to the PIMC leadership on policy matters.

Policy Review and Approval

All new or revised policies and procedures must be reviewed and approved by the Policy Committee before they can be implemented. The Policy Committee will review all policies and procedures to ensure that they are:

- Consistent with the PIMC's mission, values, and strategic plan
- Culturally competent
- Compliant with all applicable laws and regulations

The Policy Committee will provide feedback to the policy developers on any revisions that are needed. Once the Policy Committee has approved a policy or procedure, it will be submitted to the PIMC CQO for final approval.

Policy Implementation and Monitoring

PIMC will implement all approved policies and procedures in a timely manner. PIMC will monitor the implementation of all policies and procedures to ensure that they are being followed effectively. The Policy Committee will review the implementation of all policies and procedures on an annual basis.

Compliance

All PIMC employees are responsible for complying with all applicable policies and procedures. Failure to comply with a policy or procedure may result in disciplinary action.

Reference(s)

A. For both new/current Documents, the Owner is responsible for ensuring up-to-date literature references are listed.
    1. If references are appropriate and up-to-date, the Owner continues the review process
    2. If the references are not current and are no longer relevant to the subject matter, the Owner or designee is responsible for researching and recording up-to-date references

Regulatory Agency Standard(s)

The Owner or designee is responsible for researching the Document topic for regulatory standards applicable to the Document. Regulatory Agency Standards (s) are recorded in two locations for each Document. The first reference is detailed and includes the specific standard reference number. This first reference is recorded in the body of the Document under the Regulatory Agency Standards(s) header. The second reference is general and contains just the acronym for the name of the regulatory agency in the Document header.

A. The Owner collaborates with the Policy Area Owner and other Stakeholders to determine if regulatory agencies or standards listed on the Document still apply.
    1. If the agency/standard(s) are accurate and still applicable, the Owner continues the review/revision process
    2. If the references are no longer applicable, the Owner is responsible for removing and updating this information

History/Supersedes

A. The Owner is responsible for determining which related Documents are superseded by the new/revised Document and records accordingly.

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System Document Education Plan

A. Each time a Document is created/revised, the Owner collaborates with the Educational Functional Group to complete/validate the **Document Impact Assessment** in the table below, which is available for printing in the policy attachment.
B. The **Document Impact Assessment** is completed to establish the target audience's minimum education requirement(s) before posting the new/revised Document.
C. Additional education methods and competency validation may be included in the Education Plan to enhance learning.
D. The Owner completes any additional forms requested by the Education Functional Group
E. Once consensus is reached on the Document's Education Plan, the **System Document Education Plan** Form is completed, and the Owner copies and pastes the contents of this form into the Document history.
F. Note: Any policy, procedure, or plan determined to require system-wide CBL completion must be presented to the Executive Committee (EC) in the form of an SBAR and receive EC approval before implementation.

Document Impact Assessment

Impact	**Minimum Education Requirement**
[ ] Requires awareness of new/revised **non-regulatory** content And/or [ ] Requires a change in behavior or workflow to comply with new/revised **non-regulatory** content	Staff Meetings & Unit/Department Communications
[ ] Requires a significant change in behavior or workflow to comply with new/revised **regulatory** content And/or [ ] Standard/Regulation requires documentation of education	Document Acknowledgment CBL-- Brief CBL with key points highlighted and attestation of Document content review
[ ] Requires development and mastery of new skills to comply with **regulatory or non-regulatory** new/revised content And/or	Educational CBL with post-test and required passing score and/or demonstration/competency checklist.

As submitted November 29, 2023

[ ] Failure to adhere to new/revised content could result in harm to staff, patients/visitors, and/or organizational quality and/or safety

As submitted November 29, 2023